

Decision-Support Dashboard: Logic Structure

Psychiatric Discharge Continuity — Stage 1 Design

PART 1: DASHBOARD INPUTS

1A. Auto-Populated Fields

Information the system should pull from the care record without requiring case manager entry

These fields must be present before the dashboard can correctly apply workflow logic. If any are missing, the dashboard should flag the gap rather than silently proceeding with incomplete data.

Field	Source	Why Required
Patient name and care record ID	Care record	Case identification
Discharge date and time	Care record	Outreach window calculation for Metrics 2 and 3
Risk tier classification (elevated / standard)	Discharge assessment	Determines outreach timing threshold — 24–48 hr vs. 2 working days
Assigned case manager name	Discharge record	Ownership verification for Metric 6
Follow-up appointment date, time, provider, location	Scheduling system	Metric 1 confirmation baseline
Appointment confirmation status (confirmed / unconfirmed)	Scheduling system	Triggers DP1 alert if unconfirmed at discharge
Next of kin / emergency contact details	Care record	Required for structured outreach at DP3
Previous disengagement history flag	Care record	Informs risk tier accuracy check

Critical logic rule: If risk tier is absent or marked undocumented, the dashboard must default to elevated-risk pathway and generate an immediate flag. Applying standard-pathway timing to an unclassified patient is a higher-stakes error than the reverse.

1B. Case Manager–Confirmed Fields at Login or Case Opening

Fields the system can pre-populate but that require active case manager confirmation before the workflow logic activates

These exist because auto-populated data can be stale, incorrect, or incomplete. Active confirmation transfers accountability and creates a timestamped record.

- **Confirm you are the named case manager with continuity ownership for this patient** — if the logged-in user differs from the care record assignee, a mismatch alert fires immediately and routes to CM Lead (Metric 6)
 - **Confirm risk tier is accurate and current** — pre-populated from discharge assessment, but case manager must actively affirm or flag for review; an unchecked tier is treated as unverified
 - **Confirm follow-up appointment details are correct as shown** — pre-populated from scheduling system; case manager confirms or flags a discrepancy
 - **Confirm NOK / emergency contact details are current** — required before structured outreach protocol can be initiated at DP3
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1C. Case Manager–Entered or Updated Fields During Active Case Management

Information that must be entered by the case manager as coordination events occur

These are the inputs that feed real-time metric tracking and documentation completeness scoring (Metric 8). Each required entry should appear as a prompted field at the relevant workflow stage, not as a retrospective documentation task.

- **Outreach contact log:** date, time, method, outcome, next action — required at each contact attempt
- **Structured outreach initiation record:** all numbers called, alternate channels attempted, NOK contact decision and outcome — required when DP3 is triggered
- **DP3 outcome:** successful contact / unsuccessful after structured outreach — determines routing to DP4
- **DP4 escalation record:** CM Lead notified (yes/no, timestamp), treating consultant notified (yes/no, timestamp), crisis pathway activated (yes/no) — required within same business day for unreachable high-risk patients
- **Appointment attendance confirmation:** attended / missed — triggers DP5 routing
- **DP6 triage decision:** logistics barrier / clinical concern — required within 4 hours of missed appointment confirmation; must be a prompted, selected field, not a free-text note
- **DP7 continuity resolution:** restored / unresolved — required to close the active case flag; if unresolved, escalation fields activate automatically

PART 2: DASHBOARD OUTPUTS

The dashboard should load in **priority-ordered sections**, with the most urgent coordination failures and continuity risks displayed first and requiring acknowledgment before the case manager scrolls to lower-priority content. The structure below reflects that priority sequence.

OUTPUT SECTION 1 — Critical Alerts Panel

Requires acknowledgment before proceeding

What it displays: A compact, high-visibility panel listing every active case in the caseload that has breached a hard threshold — meaning a required action has not occurred within its defined window, or a required documentation record is absent. Each alert names the patient, the specific failure, the time elapsed since breach, and the required next action. Alerts are not grouped by metric — they are grouped by urgency and presented as explicit action items.

Examples of what appears here:

- *"Patient [X] — elevated risk — no contact documented 52 hours post-discharge. Same-day escalation to CM Lead required. Escalation not yet recorded."*
- *"Patient [Y] — missed appointment confirmed 6 hours ago. DP6 triage decision not documented. Required within 4 hours. Flag sent to CM Lead."*
- *"Patient [Z] — ownership assignment: mismatch detected between care record and logged-in user. CM Lead notified."*

Metrics drawn from: 2 (outreach timeliness), 3 (structured outreach escalation), 4 (missed appointment same-day review), 6 (ownership assignment), 8 (documentation completeness)

Coordination failure it catches: Any breach of the workflow's hard timing requirements — the points where delay most directly translates to patient risk and where the workflow explicitly prohibits silent failure.

Role responsible: Case manager for action; CM Lead automatically notified for any alert that has been active for more than 2 hours without a documented response.

How it supports rapid decision-making: The case manager opening the dashboard at the start of a busy day does not need to scan a full caseload view to identify what is most urgent. Critical alerts are front-loaded, named, and tell the case manager exactly what to do. Acknowledgment is required — the case manager cannot mark an alert as seen without selecting a response action or logging a note, which simultaneously satisfies the documentation requirement.

OUTPUT SECTION 2 — Active Outreach Window Tracker

Time-sensitive but not yet breached

What it displays: A list of all patients currently within their active outreach window — meaning discharge has occurred, the outreach window has opened, but the deadline has not yet passed. Each patient row shows: risk tier, discharge date/time, outreach window deadline, time remaining, and current contact status (not yet attempted / attempted, no contact / contact confirmed). Patients approaching their deadline within the next 4 hours are visually differentiated from those with more time remaining.

Metrics drawn from: 2 (outreach timeliness by risk tier), 6 (ownership assignment)

Coordination failure it catches: Outreach delays that have not yet breached the threshold but are at risk of doing so — particularly important on high-volume days when a case manager may have multiple patients in overlapping outreach windows. This section surfaces approaching failures before they become critical alerts.

Role responsible: Assigned case manager for each patient row.

How it supports rapid decision-making: Rather than requiring the case manager to remember which patients need contact and by when, the tracker makes the full active outreach obligation visible in a single view, ordered by time remaining. The case manager can work through the list in priority order without reconstructing timelines from memory or care record review.

OUTPUT SECTION 3 — Pre-Discharge Confirmation Status

Retrospective flag and forward-looking gate

What it displays: For any patient discharged within the last 24 hours, this section confirms whether the appointment was confirmed before discharge, whether the case manager ownership record is complete, and whether the continuity ownership notification was documented. For any patient where one of these elements is missing, the section displays a specific gap and a prompted resolution action — for example, a direct link to the scheduling system to confirm the appointment, or a prompted field to document ownership.

It also displays the rolling weekly pre-discharge confirmation rate across the case manager's caseload, so trends are visible without requiring a separate report.

Metrics drawn from: 1 (pre-discharge appointment confirmation rate), 6 (ownership assignment gap)

Coordination failure it catches: Patients who have already exited the inpatient setting without a confirmed appointment or documented ownership — the foundational continuity failure that increases downstream coordination risk for every subsequent workflow step.

Role responsible: Case manager for resolution; CM Lead visibility if gap is unresolved more than 4 hours post-discharge.

How it supports rapid decision-making: By surfacing this immediately after discharge rather than waiting for a missed outreach or appointment, the dashboard allows the case manager to correct the gap while the patient is still reachable and the scheduling window is open — rather than discovering the problem only when outreach fails.

OUTPUT SECTION 4 — Missed Appointment Response Tracker

Active resolution monitoring

What it displays: All patients who missed their first follow-up appointment and are currently in the recovery pathway — organized by stage. Each patient row shows: time since missed appointment, whether same-day review was completed, the DP6 triage decision recorded (or a flag if absent), which resolution pathway was activated (reschedule / risk reassessment / crisis), and current DP7 status (continuity restored / unresolved). For unresolved cases, the row displays the next required escalation action and the time remaining before that action becomes overdue.

Metrics drawn from: 4 (missed appointment same-day review), 5 (resolution pathway completion), 8 (documentation completeness)

Coordination failure it catches: Cases stalled between DP5 and DP7 — where the missed appointment was detected but the recovery pathway was not completed, or where the DP6 triage decision was made informally but not documented, leaving the resolution invisible to supervision and handoff.

Role responsible: Case manager for active pathway steps; CM Lead for any case where DP7 shows unresolved and no escalation is documented.

How it supports rapid decision-making: Missed appointment recovery involves multiple sequential steps across a compressed timeframe. This section makes the full recovery sequence visible as a tracked pipeline rather than a series of individual tasks the case manager must remember independently. The case manager can see at a glance which cases are progressing and which are stalled.

OUTPUT SECTION 5 — Documentation Completeness Prompt Panel

Embedded throughout, consolidated here

What it displays: A summary of all open documentation requirements across the active caseload — specifically, required fields that have not yet been completed for cases currently in an active workflow stage. This is not a historical audit panel. It displays only current, actionable gaps: outreach attempt records not yet completed, DP6 decisions not yet entered, escalation confirmation records pending, DP7 resolution fields not yet closed.

Each item links directly to the relevant field so the case manager can complete it in place, with a timestamp automatically applied on submission.

Metrics drawn from: 8 (documentation completeness for escalation actions) — draws on escalation records from Metrics 3, 4, and 5

Coordination failure it catches: The gap between actions taken and actions recorded — specifically the risk that a case manager completes a coordination step but defers documentation until later, creating a window during which the case is invisible to supervisors, handoff recipients, and the dashboard's own logic.

Role responsible: Case manager for completion; CM Lead receives a summary of open documentation gaps across the team at end of day.

How it supports rapid decision-making: By surfacing documentation requirements as a prompted, consolidated list rather than leaving them as implicit expectations, the panel reduces the cognitive overhead of remembering what needs to be recorded while also eliminating the need for the case manager to navigate separately to each record. Documentation becomes a task that fits within the dashboard workflow rather than competing with it.

OUTPUT SECTION 6 — Caseload Load and Escalation Visibility Summary

Supervisory awareness and workload context

What it displays: A single-screen summary of the case manager's full active post-discharge caseload: total patients in active monitoring, number in elevated-risk pathway, number in standard pathway, number with open critical alerts, number with outreach overdue, number with unresolved missed appointment recovery, and current documentation completeness percentage. This view is visible to both the case manager and the CM Lead.

For the CM Lead, this section aggregates across all case managers on the team, allowing rapid identification of individuals carrying disproportionate risk volume or accumulating overdue actions — the team-level coordination risk that no individual case manager can see from their own dashboard view.

Metrics drawn from: 7 (caseload concentration and outreach overdue volume), cross-references all other metrics

Coordination failure it catches: Risk concentration that exceeds individual case manager capacity without generating a visible signal to supervision — the structural gap where workflow compliance depends on individual vigilance rather than system-level awareness.

Role responsible: Case manager for awareness of own load; CM Lead for team-level monitoring and redistribution decisions.

How it supports rapid decision-making: When a case manager opens the dashboard on a high-volume day and immediately sees that they have four elevated-risk patients in active outreach windows, two open critical alerts, and a documentation completeness score of 78%, they have an accurate operational picture before they open a single patient record. That visibility supports prioritization decisions that would otherwise require manual caseload review.